National Cancer Control Programme (NCCP) 00269 · clinician draft

FEC50

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FEC50 · treatment-day support

Active medicines: Fluorouracil (5-FU) + Epirubicin + Cyclophosphamide

Emetogenic risk: High · anthracycline / cyclophosphamide

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 National Cancer Control Programme (NCCP) anthracycline–cyclophosphamide row: no routine Days 2–4 dexamethasone; local/prescriber review applies.
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Review granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) need with the consultant; FEC count/FN tables include specific alternatives and daily G-CSF Days 4–11 where selected. Pegylated products need their own timing.
- Confirm an individual hydration plan; the source Day 1 fluid advice must be adapted for cardiac or renal restrictions.
- Fertility, contraception, extravasation and urgent-contact advice are part of the treatment discussion.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00269 v8 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-23 · SACTCheck v0.84.17 · Regimen-level drafts; prescriber and oncology-pharmacy verification required